PC-04 Pediatric Pulseless Arrest

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | OCTOBER 26, 2026

Scope: Immediate routing for a pulseless patient age 3 days through 15 years. Use AO-02 Newly Born through 3 days and the adult cardiac-arrest pathway at age 16 years or older.

Field flow

1. Confirm	Pulselessness, age pathway, DNR/MOST, and obvious-death criteria.
2. CPR + BVM	100-120/min, depth at least one third chest AP diameter, full recoil, pauses under 10 sec. Use 30:2 with one rescuer or 15:2 with two.
3. AED / monitor	Apply immediately. Use pediatric pads/attenuator when available, but do not delay defibrillation if standard equipment is immediately available.
4. Rhythm route	VF/pulseless VT: PC-07. Asystole/PEA: PC-01. Obtain IV/IO without delaying CPR or ventilation.
5. ROSC	Use PC-08 and notify the pediatric-capable destination early.

Provider-level actions

Level	Approved actions
EMT	High-quality CPR, BVM ventilation, AED/monitor, rhythm-specific routing, length-based system, and minimal interruptions.
AEMT	All EMT care plus prompt IV/IO, glucose/temperature assessment, authorized naloxone, and advanced airway support within standing orders.
Paramedic	All prior care plus rhythm-specific manual treatment, waveform EtCO2 after advanced airway, reversible-cause management, and post-ROSC transition.

PC-04 Pediatric Pulseless Arrest

SAFETY, MEDICATION, AND LINKED-PATHWAY REFERENCE

Safety and escalation

- Naloxone does not replace CPR and ventilation.
- Do not delay CPR, ventilation, or defibrillation for IV access, medication, or intubation.
- Do not pause CPR after a shock to check a pulse; resume until the next planned rhythm check.
- Use waveform EtCO2 trends to improve CPR and identify possible ROSC; do not use a single value for termination.

Medication reference

Medication	Approved dose / safeguard
Naloxone	Suspected opioid-associated arrest: 0.01 mg/kg IV/IO/IM/IN initially; if needed 0.1 mg/kg; max 2 mg per dose and 8 mg cumulative. Continue CPR and ventilation.

Linked pathways

AR-05 Pediatric Airway; AR-06 Pediatric Failed Airway; PC-01 Asystole / PEA; PC-07 VF / Pulseless VT; PC-08 Post-Resuscitation.

Source control

- This controlled direct source PDF replaces the prior source file and matches approved Claiborne EMS app content released October 26, 2026.
- Use current Claiborne standing orders, formulary, equipment authorization, and Medical Control direction whenever they govern a case-specific decision.